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PRECISION DIAGNOSTICS & AI ANALYSIS

LABINTEL DIAGNOSTICS SYSTEMS

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Regional Diagnostics Hub, Kolkata - 700157

NABL ACCREDITED LAB

PATIENT NAME

SAYANTAN MAJI

PATIENT ID (PID)

LAB-123

GENDER / AGE

male / 21 Yrs

REFERRING PHYSICIAN (REF. DR.)

DR. ROY (RECOVERED)

COLLECTION DATE

02 May 2026

REPORT REF	#A59BE841
RELEASE DATE	02 May 2026
STATUS	FINAL

SPECIALIST RECOMMENDED

Clinical Pathologist

NEURAL CLINICAL INTERPRETATION

BIOMARKER CORRELATION ENGINE V4.0

AI SYNTHESIS SUMMARY

AI Analysis detected 3 significant deviation(s) in your profile. Specifically, Thrombocytopenia (Low Platelets), Blood Viscosity Shift, Microcytic Indices, suggest(s) a need for targeted clinical investigation. Rule out splenic sequestration or marrow suppression. Assess hydration status and erythropoiesis. Serum Ferritin and TIBC correlation. Immediate consultation with

90

VITALITY INDEX

GOLD STANDARD STABILITY

a Clinical Pathologist is recommended to prevent further progression.

CONFIDENCE
96.8% Match

RISK LEVEL
ELEVATED

PREMIUM HEALTH TIER

INVESTIGATION	RESULT	REFERENCE VALUE	UNIT
Hemoglobin	16 G/DL	13 - 17	NORMAL
AI CLINICAL INSIGHT: Optimal oxygen-carrying capacity detected.			
RBC Count	4.5 M/ML	4.5 - 5.5	NORMAL
AI CLINICAL INSIGHT: Marker is within optimal physiological range.			
WBC Count	10000 /ML	4500 - 11000	NORMAL
AI CLINICAL INSIGHT: Marker is within optimal physiological range.			
Platelets	20000 /ML	150000 - 400000	CRITICAL_LOW
AI CLINICAL INSIGHT: Critical risk of clotting dysfunction. Requires immediate hematological review.			
Hematocrit	70 %	40 - 52	CRITICAL_HIGH
AI CLINICAL INSIGHT: High viscosity detected. Suggests dehydration or RBC overproduction.			
MCV	70 FL	80 - 100	CRITICAL_LOW

INVESTIGATION	RESULT	REFERENCE VALUE	UNIT

AI CLINICAL INSIGHT: **Suggests impaired oxygen transport and possible iron deficiency pattern.**

AI DIAGNOSTIC ROADMAP

MAJOR CLINICAL CONDITION IDENTIFIED

THROMBOCYTOPENIA (LOW PLATELETS)

AI SOLVING PROCESS (NEXT 14 DAYS)

1. Immediate hydration and electrolyte stabilization.
2. Initiate Ferrous Ascorbate therapeutic protocol.
3. Repeat biomarker analysis for correlation check.

REQUIRED AI-SUGGESTED THERAPY

FERROUS ASCORBATE

1 TAB / ONCE DAILY / POST-CIBUS

EXPECTED OUTCOME

Optimization of biological reference ranges and systemic recovery.

AI CLINICAL INTERPRETATION

- **SEVERE THROMBOCYTOPENIA**
  Elevated risk of spontaneous mucosal or systemic bleeding. Requires immediate monitoring.
- **HYPERVISCOSITY RISK**
  Elevated RBC concentration suggests possible dehydration or secondary polycythemia.
- **MICROCYTOSIS DETECTED**
  Suggests impaired hemoglobin synthesis, common in Iron Deficiency Anemia.

PRIORITY RECOMMENDED TESTS

- Repeat CBC (within 24–48 hrs)
- Peripheral Blood Smear (PBS) Correlation
- Iron Profile (Serum Ferritin, TIBC, UIBC)
- Serum Vitamin B12 & Folate levels

RECOMMENDED SPECIALIST TYPE

Clinical Pathologist

- Expertise required for Thrombocytopenia (Low Platelets) management and diagnostic correlation.

CONSULT EXPERT NOW

LABORATORY INSTRUMENTATION

Automated Hematology Analyzer v5.0 (Laser Flow Cytometry)

INTERNAL QUALITY CONTROL: PASSED

CERTIFIED TECHNICIAN

Ananya Bose
SENIOR LAB TECHNOLOGIST

BIOLOGICAL REFERENCE INTERVALS

Reference ranges are derived from standardized clinical cohorts and validated for male adults.

END OF CLINICAL REPORT

MEDICAL DISCLAIMER

This report is generated with AI assistance and is for informational purposes only. It does not constitute a formal medical diagnosis or prescription. All findings MUST be reviewed and confirmed by a certified medical professional before clinical decisions are made.

CHIEF PATHOLOGIST
MD Path / NABL Signatory

DIGITAL FINGERPRINT
B9WFK9D4HA8-OVABTWGMGE

DIGITALLY SECURED & AI VERIFIED

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